

[SYNTHETIC SAMPLE] Fictional record generated for the DELPHi prototype demo. Names, institutions, and statements are invented; the layout imitates a generic field-medical format. Not a real document and not derived from any real record.

[CONFERENCE REFERENCE] The meeting named above is a real, publicly announced scientific congress and is used here only as the setting of this fictional record. No session content, abstract, poster, or presented data from that meeting is reproduced or paraphrased; every person and conversation below is invented.

Congress Attendance Report — American Epilepsy Society Annual Meeting (AES) 2022

Meeting period: December 2022 | Author: Marcus Obeng

Scope: unsolicited conversations held on-site. Session content and presented data are deliberately not reproduced here.

Conversation — Walter Brandt, MD (Flint Hills Neuroscience)

A hallway conversation during afternoon break. Described practice as early in its experience here, with nothing to report yet. Titration scheduling is handled by nursing staff, which practice finds workable.

Conversation — Valentina Danforth, MD (Beacon Hill Neurology Associates)

Caught between sessions in the poster hall; the convo was short and entirely unprompted. Described how refractory group is triaged now compared with before. Described preference for slower titration in general, unrelated to any particular pt.

Conversation — Dagmar Lindenmuth, MD (Alder Point Neurology Associates)

We spoke standing near exhibit aisles, briefly. Referral patterns into this clinic have changed, and long-standing pts is a bigger share than it was. Referenced review that circulated on service in passing.

Conversation — Mateo Zeitler, MD (Northbrook Neurology Institute)

Caught between sessions in poster hall; convo was short and entirely unprompted. Titration scheduling is handled by nursing staff, which the practice finds workable. Referenced review that circulated on service in passing. Nothing that needed f/u this time. Asked about safety and confirmed nothing to report.

Conversation — Bettina Marchetti, MD (Quarry Hill Epilepsy Center)

A short exchange over the coffee queue. Asked whether I had seen a guideline update discussed at journal club; I had not, and offered nothing in return. Benefit verification is the step that most often delays a start here.

Conversation — Viraj Adeyemi-Cole, MD (Thornfield Regional Epilepsy Program)

Approached me rather than other way around, which is worth noting. Compared notes on which sessions were worth the time. Mentioned review that circulated on service but did not raise any specific finding. No specific request. Safety items asked about and confirmed none.

Conversation — Eloise Quintrell, MD (Beacon Hill Neurology Associates)

A short exchange over the coffee queue. Benefit verification is the step that most often delays start here. This practice is still building familiarity with the product and had no clinical points to raise. Compared notes on which sessions were worth time.

Conversation — Bram Merriwether, MD (Willowmere Neuroscience Center)

A short exchange over coffee queue. Nothing that needed f/u this time. Asked about safety and confirmed nothing to report. Referenced a guideline update discussed at journal club in passing. Mostly small talk about program and venue.

Conversation — Rasmus Galbraith, MD (Marbury Neurological Care Center)

We spoke standing near the exhibit aisles, briefly. Nothing actionable this visit this time. Asked about safety and confirmed nothing to report. Spent some time on the surgical workup list and how workup decisions get sequenced.

Conversation — Harold Jessup, MD (Magnolia Grove Clinic)

Introduced by colleague from same region and talked for few minutes. Talked through newly referred patients in general terms, without any specific case. Benefit verification is the step that most often delays start here. Titration scheduling is handled by nursing staff, which practice finds workable.

Follow-ups have been logged in the interaction system.